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CASE NAME: Tambor, et al., v. Skirball Hospice, et al.

CASE NO.: 24SMCV02741

MOTION: Demurrer and Motion to Strike the Third Amended Complaint

HEARING DATE: 9/22/2026

LEGAL STANDARD

A demurrer for

sufficiency tests whether the complaint states a cause of action. (Hahn v. Mirda (2007) 147 Cal.App.4th 740, 747.) When considering demurrers, courts read the allegations liberally and in context. In a demurrer proceeding, the defects must be apparent on the face of the pleading or via proper judicial notice. (Donabedian v. Mercury Ins. Co. (2004) 116 Cal.App.4th 968, 994.) A demurrer tests the pleadings alone and not the evidence or other extrinsic matters. Therefore, it lies only where the defects appear on the face of the pleading or are judicially noticed. (CCP §§ 430.30, 430.70.) At the pleading stage, a plaintiff need only allege ultimate facts sufficient to apprise the defendant of the factual basis for the claim against him. (Semole v. Sansoucie (1972) 28 Cal. App. 3d 714, 721.) A “demurrer does not, however, admit contentions, deductions or conclusions of fact or law alleged in the pleading, or the construction of instruments pleaded, or facts impossible in law.” (S. Shore Land Co. v. Petersen (1964) 226 Cal.App.2d 725, 732, internal citations omitted.)

A special demurrer for

uncertainty is disfavored and will only be sustained where the pleading is so bad that defendant cannot reasonably respond—i.e., cannot reasonably determine what issues must be admitted or denied, or what counts or claims are directed against him/her. (CCP § 430.10(f); *Khoury v. Maly's of Calif., Inc.* (1993) 14 Cal.App.4th 612, 616.) Moreover, even if the pleading is somewhat vague, “ambiguities can be clarified under modern discovery procedures.” (Ibid.)

“Liberality in

permitting amendment is the rule, if a fair opportunity to correct any defect has not been given.” (*Angie M. v. Superior Court* (1995) 37 Cal.App.4th 1217, 1227.) It is an abuse of discretion for the court to deny leave to amend where there is any reasonable possibility that plaintiff can state a good cause of action. (*Goodman v. Kennedy* (1976) 18 Cal.3d 335, 349.) The burden is on plaintiff to show, in what manner, plaintiff can amend the complaint, and how that amendment will change the legal effect of the pleading. (Id.)

ANALYSIS

Defendant Faith and

Hope Home Health Inc. demurs to Plaintiffs Rina Tambor and Matthew Tambor's Third Amended Complaint (“TAC”). Defendant moves to strike the requests for attorneys' fees and punitive damages in connection with the elder abuse cause of action.

Medical

Negligence

Defendant argues that the first

cause of action for medical negligence (survival damages) fails to state sufficient facts. The elements of a professional negligence/medical malpractice cause of action are: “(1) the duty of the professional to use such skill, prudence, and diligence as other members of his profession commonly possess and exercise; (2) a breach of that duty; (3) a proximate causal connection between the negligent conduct and the resulting injury; and (4) actual loss or damage resulting from the professional's negligence.” (*Borrayo v. Avery* (2016) 2 Cal.App.5th 304, 310.) “Ordinarily, negligence may be alleged in general terms, without specific facts showing how the injury occurred, but there are ‘limits to the generality with which a plaintiff is permitted to state his cause of action, and . . . the plaintiff must indicate the acts or omissions which are said to have been negligently performed.’ He may not

recover upon the bare statement that the defendant's negligence has caused him injury.' [Citation]." (Berkley v. Dowds (2007) 152 Cal.App.4th 518, 527.) However, there is no requirement that plaintiff identify and allege the precise moment of the injury or the exact nature of the wrongful act. (Hahn v. Mirda (2007) 147 Cal.App.4th 740, 747.)

The TAC states a claim for medical malpractice against Defendant. The TAC concerns the death of Yona Tambor. (TAC ¶ 5.) Plaintiffs are Decedent's heirs and successors-in-interest. (Id., ¶¶ 14, 17.) The events giving rise to the claims occurred from April 15, 2022, through decedent's death on March 25, 2023. (Id., ¶ 4.) Defendant held itself out as a health care provider licensed and authorized to practice in California. (Id., ¶¶ 7-9.) Defendant sent health care personnel or otherwise provided home health care and hospice care to Decedent. (Id., ¶ 10.) Defendant caused damages Decedent by "negligently, wantonly, recklessly, tortiously and unlawfully:

A. Took a history, examined, tested, diagnosed, cared for, treated, advised, supervised, handled and managed [Decedent];

B. Managed, maintained, operated, supervised, selected and controlled the health care providers, practitioners of the healing arts and its staff and equipment;

C. Provided health care, nursing and attendant care to [Decedent];

D. Failed to inform, explain, warn and advise [Decedent] of the dangers, hazards, risks and complications of the various health care procedures, care and treatment to be performed;

E. Failed to guard and protect [Decedent];

F. Withheld from [Decedent] facts and information necessary to enable [Decedent] to form an intelligent, free, real and informed consent to the procedures, care and

treatment, of which facts and information [Decedent] was unaware; and

G.

Conducted [itself] with reference to [Decedent] so as to proximately cause, permit and allow [Decedent] to suffer medical harm, injury and damages, as a proximate result of said negligence."

(Id., ¶ 11.) The TAC further alleges that during the time period of April 15, 2022, and March 25, 2023, as a proximate result of the negligence of Defendant, Decedent suffered pain and suffering. (Id., ¶ 15.) As a result, Decedent required medical and related health care services. (Id., ¶ 16.)

The TAC states sufficient facts to support a medical negligence cause of action and therefore, the demurrer is OVERRULED.

Elder Abuse

Defendant

argues that the cause of action for Dependent Adult Abuse fails to state a cause of action under Welfare & Institution Code §15610 et. seq. because there are no allegations that it had a substantial caretaking or custodial relationship with Decedent. Further, Defendant contends that the TAC fails to allege facts evidencing reckless conduct on its part.

To

establish a claim for Elder Abuse and Neglect under the Act, a plaintiff must allege the following: (1) plaintiff is an elder or dependent adult; (2) defendant had responsibility for meeting the basic needs of the elder or dependent adult, such as nutrition, hydration, hygiene or medical care; (3) defendant knew of conditions that made the elder or dependent adult unable to provide for his or her own basic needs; (4) defendant denied or withheld goods or services necessary to meet the elder or dependent adult's basic needs, either (a) with knowledge that injury was substantially certain to befall the elder or dependent adult (if the plaintiff alleges oppression, fraud or malice) or (b) with conscious disregard of the high probability of such injury (if the plaintiff alleges recklessness); and (5) the neglect caused the elder or dependent adult to suffer physical harm, pain or mental suffering. (Carter v. Prime Healthcare Paradise Valley LLC (2011) 198 Cal.App.4th 396, 407; Welf. & Inst. Code, § 15610.07.) Elder abuse must be pleaded with

particularity in accordance with the pleading rules governing statutory claims.

(Ibid.)

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First, Defendant contends that the TAC

does not allege it had a substantial caretaking or custodial relationship.

“Neglect” within the meaning of section

15610.57 covers an area of misconduct distinct from professional negligence. (Covenant

Care, Inc. v. Superior Court (2004) 32 Cal.4th 771, 783.) “As used in the

Act, neglect refers not to the substandard performance of medical services but,

rather, to the ‘failure of those responsible for attending to the basic needs

and comforts of elderly or dependent adults, regardless of their professional

standing, to carry out their custodial obligations.’” (Id.) The Act

“does not apply unless the defendant health care provider had a substantial

caretaking or custodial relationship, involving ongoing responsibility for one

or more basic needs, with the [] patient.” (Winn v. Pioneer Medical Group,

Inc. (2016) 63 Cal.4th 148, 152.) “It is the nature of the elder or

dependent adult’s relationship with the defendant—not the defendant’s

professional standing—that makes the defendant potentially liable for neglect.”

(Id.) A caretaking or custodial relationship only occurs where a party

“has assumed a significant measure of responsibility for attending to one or

more of an elder’s basic needs that an able-bodied and fully competent adult

would ordinarily be capable of managing without assistance.” (Id. at

158.)

The TAC alleges a robust and/or

ongoing caretaking or custodial relationship beyond acts incidental to

circumscribed medical care. According to the TAC, Defendant, through its managing

agents and employees, had the care, custody or control and/or stood in a

position of trust with the Decedent, and had a caretaking and custodial

relationship with Decedent. (TAC ¶¶ 21-22.) Decedent

was entirely dependent on Defendant’s staff and affiliated providers for her

daily needs, including mobility assistance, incontinence care, repositioning,

nutrition, hydration, medical attention, hygiene, and general supervision.

(Id., ¶ 23.) The cases cited by Defendant were not resolved on the pleadings,

and the evidentiary records in those cases demonstrated no custodial

relationship. (See Oroville Hospital v. Superior Court (2022) 74

Cal.App.5th 382, 388-392, 405-406 [at summary judgment, no caretaking or

custodial relationship existed where defendants assumed home health services for

wound care, and decedent relied on her granddaughter all other aspects of her care; wound care could neither be considered a “basic need”]; *Kruthanooch v. Glendale Adventist Medical Center* (2022) 83 Cal.App.5th 1109, 1118 [JNOV was proper where no evidence at trial supported a robust and ongoing caretaking relationship between an emergency room operator and patient; claims arose from burns patient sustained in the MRI machine caused by ECG pads negligently left on patient's body, the relationship was of a limited duration and any attention to the patient's basic needs was incidental to the circumscribed medical care it provided].) Defendant also argues that it provided only non-custodial care to decedent for a matter of days, from April 22, 2022, to April 28, 2022. Defendant cites no basis for this contention. To the contrary, the TAC alleges that the events giving rise to the claims occurred from April 15, 2022, through decedent's death on March 25, 2023. (TAC ¶¶ 4, 15.)

Next, Defendant argues that the allegations do not show reckless conduct on its part. As noted, a plaintiff must allege that defendant denied or withheld goods or services necessary to meet the elder or dependent adult's basic needs, either: (a) with knowledge that injury was substantially certain to befall the elder or dependent adult; or (b) with conscious disregard of the high probability of such injury. (*Carter*, supra, 198 Cal.App.4th at 407.) This requirement can be understood as constituting an “egregiousness” standard for Elder Abuse liability. (*Id.* at 405.) The egregious withholding of medical care can, depending on the facts alleged, constitute actionable neglect for the purposes of an elder abuse claim. (*Id.*, at 405-406 [compiling several example cases]; *Covenant Care, Inc.*, supra, 32 Cal.4th at 786; see, e.g., *Fenimore v. Regents of the University of California* (2016) 245 Cal.App.4th 1339, 1349-50 [while lone allegations of understaffing and undertraining leading to injury are insufficient to arise to recklessness, they can demonstrate recklessness when there is a showing of a knowing pattern of violations of staffing regulations].)

The TAC alleges Decedent suffered from avoidable pressure ulcers and other injuries while under the care of Defendants, which would not have developed absent a pattern of custodial neglect. (TAC ¶ 24.) The injuries were caused by Defendant's failure to reposition Decedent, monitor her condition, provide adequate hygiene, or ensure she received appropriate nutrition and hydration. (*Id.*) Defendant “willfully, intentionally, and/or recklessly caused or permitted Decedent to be injured and/or to be placed in a situation such that her health was in danger.”

(Id., ¶ 25.) Defendant “actually or constructively” knew that Decedent was “at high risk for pressure ulcers and other preventable complications due to her medical condition and immobility, yet failed to take the necessary steps to protect her from these foreseeable harms.” (Id., ¶ 26.) Defendants “created circumstances or conditions likely to produce great bodily harm” and “willfully caused or permitted Decedent to suffer, and inflicted upon her, unjustifiable physical pain, injuries, damages, suffering and indignity.” (Id.) Defendant deprived Decedent of needed medical care at home, failed to ensure that said staff was adequately trained, and failed to properly treat and care Decedent, among other things. (Id., ¶ 27.) Defendant failed to exercise that degree of care that a reasonable person would have exercised in the care of Decedent in their willful and deliberate failure to provide to Decedent needed medical care, including their failure to appropriately treat or care for Decedent in light of her bed sores and the likelihood that she would suffer severe injuries from such, to adequately provide Decedent care in accordance with her plan of care, and other deficiencies all which led to Decedent’s serious injuries. (Id., ¶ 28.) As a result of this neglect, Decedent developed bedsores and other injuries. (Id., ¶¶ 29, 31.) Defendant is guilty of recklessness, oppression, and/or malice by due to its willful and intentional failure to provide Decedent with needed medical care. (Id., ¶¶ 30, 32.)

In summary, the TAC alleges that Defendant willfully failed to reposition Decedent, provide her hygiene, or ensure she received appropriate nutrition/hydration despite known risks of pressure ulcers. These facts suggest that Defendant acted with conscious disregard of the high probability of such injuries. Caselaw recognizes that failing to provide a patient with sufficient nutrition/hydration and leaving a patient unattended/unmonitored, despite a known risk of pressure ulcers, could be considered neglect under the statute. (Carter, supra, 198 Cal.App.4th at 405-406; Covenant Care, supra, 32 Cal.4th at 778 [failure to provide an elderly man suffering from Parkinson’s disease with sufficient food and water and necessary medication, leaving him unattended and unassisted for long periods of time in his own excrement so that ulcers exposing muscle and bone became infected]; Delaney v. Baker (1999) 20 Cal.4th 23, 27, 41 [frequently left patient lying in her own urine and feces for extended periods of time, developed pressure ulcers despite plaintiff’s persistent complaints to staff]; Sababin v. Superior Court (2006) 144 Cal.App.4th 81, 83–87, 90 [facility ignored a medical care plan requiring the facility to check the dependent adult’s skin on a daily basis and failed to notify a physician when pressure ulcers and other skin lesions developed].)

Accordingly,
the demurrer is OVERRULED.

Motion to Strike

In the motion to strike, Defendant asserts that the TAC fails to establish recklessness under the Elder Abuse Act, and therefore, their claim for attorney fees and costs and punitive damages should be stricken. As discussed above, the TAC establishes sufficient facts for imposition of the enhanced remedies under the Act. Accordingly, the motion is DENIED.